



Policy

Clinical Companion and Patient Special Management

Scope

Site	Service/Department/Unit	Disciplines
SCGOPHCG	Organisation Wide	All Staff

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Introduction

Any patient requiring a clinical patient special for close observation is subject to the application of this policy. This policy applies to all classifications of nurses to special an inpatient at Sir Charles Gairdner Osborne Park Health Care Group SCGOPHCG. The decision to special a patient can only be requested and documented by an appropriate Senior Registered Nurse (SRN).

Risk Statement

Non-compliance with this policy may:

Breach legislative requirements	☐	Impact on Patient Quality of Care	☒
Breach National/State/Hospital Policy	☒	Impact on Patient Safety	☒
Breach professional standards	☐	Misconduct	☐
Breach SCGOPHCG Mission & Values	☒	Staff Safety	☐
Other:	☐		

Definitions

At Risk	A patient whose physiological condition/cognition and/or mental state is deemed to put them at heightened risk of clinical deterioration and/or being unsafe and/or experiencing an adverse event (actual or potential harm).
Acute Severe Behavioural Disturbance (ASBD) / Behaviours of Concern (BoC)	Can be used interchangeably with Acute Behavioural Disturbance (ABD) and puts the patient or others at risk of harm and may include threatening or aggressive behaviour, extreme agitation or distress, self-harm which could cause injury or death.
Increased levels of observations staff: patient ratio	• 2:1 (2 staff to 1 patient). • 1:1 (1 staff to 1 patient). • 1:2 (1 staff to 2 patients). • 1:3 (1 staff to 3 patients). • 1:4 (1 staff to 4 patients). Complex care and infection control requirements must be considered in any specialising arrangement.
Indicators of acute mental state deterioration, including delirium	• Reported change in cognition by the patient or someone who knows them well. • New confusion • Level of distress. • Loss of touch with reality or understanding consequences of behaviour. • Loss of physical function. • Change in behaviour • Imminent risk to self, others, or property.
Indicators of physiological deterioration	• Physiological changes that may indicate a decline of the patient's health status, e.g. - Injury to the body requiring care greater than general first aid - Respiratory distress (fast, slow or irregular breathing) - Sweating more than normal - Dizziness - Vomiting - Nausea - Chest pain and/or new pain and/or uncontrolled pain - Seizure - Collapse - Loss of consciousness

Security Special	• A security officer may be employed to provide assistance in management of patients with ASBD. • All NMHS security have certification in restrictive practice. Agency security officers will be able to guide and assist with care, however are not certified to use restrictive practices.
Staff who can conduct a “special”	• A registered nurse (RN), enrolled nurse (EN), assistant in nursing (AIN) or Patient Care Assistant (PCA) employed to provide close visual observation and/or physical proximity to prevent/reduce risk and /or manage a possible adverse event, and to assist in the provision of total patient care within their scope of practice and classification • Is accountable to the ward SRN in the area they are allocated or after hours SRN.
Specialling	• A designated staff member allocated to provide a close level of observation and care for a defined period of time. • There may be circumstances where the ratio of staff to patient will vary - this will be addressed on an individual basis following clinical assessment. • Students may not undertake the role of a special.
Ward or Afterhours SRN	Clinical Nurse Specialist (CNS) Clinical Nurse Manager (CNM) and Afterhours CNS (AHCNS).

Policy Principles

Staff have a duty of care to maintain the safety and well-being of patients while promoting a safe, supportive, and least restrictive environment. Observation and engagement with the patient are one of many interventions that may help to maintain the patient's safety. Observation of patients should be undertaken sympathetically and only when necessary.

The least intrusive and least restrictive level of observation that is appropriate to the situation should always be adopted.

What keeps people safe is not the act of being under surveillance (observation); it is the quality of engagement between that individual and staff.

1. Identifying Patients Requiring Increased Levels of Observation

Specialling and closer levels of observation may be required for a patient assessed as at risk of:

- Clinical deterioration
- Acute delirium, mental health or behavioural disturbance (including self-harm) and known or suspected cognitive impairment.
- Potential injury to self - e.g., patients deliberately harming their bodies, patients interfering with treatment interventions such as the removal of invasive lines, requiring regular re-orientation and diversion communication etc
- Falls where falls prevention strategies have been unsuccessful.
- Absconding or wandering
- Patients who are at significant risk to self or others, damage to reputation, including violence and aggression. Consideration must be given to maintaining a safe environment and distance to minimise the risk to staff.
- Harm to others via transmission of infection
- If patient is placed under the MHA 2014, ideally a Mental Health Nurse Special or a Registered Nurse 1:1 (where available) is to be provided. If patient is admitted from

Immigration or Custodial Services, they may have guards in place to prevent them absconding from hospital site.

- The patient normally has a carer in the community.

Increased levels of observation may not be required if alternative strategies are implemented to achieve an improved level of safety and significantly lower risk - which may include but are not limited to:

- Revised staff-patient allocation
- Revised room allocation
- Reduced stimuli
- Addressing basic healthcare needs e.g., pain, hunger, toileting
- Use of family members/patient support person(s) (PSPs) or carers, either in person or via audio visual aids
- Involving Aboriginal Health Liaison Officer and/or Social Worker, Clinical Psychologist or referral to Mental Health Liaison Service
- Other risk minimisation strategies (e.g., medication, falls/alarm mats, ultra-low beds, known triggers/relievers).

Alternative patient management strategies must be clearly documented in the patient's integrated notes/care plan/Behaviour Observation Form (BOF).

- The treating team and/or psychiatry team for review of treatment regimens, medications, and observation categories.
- Use clinical discretion if the nursing special should be inside or outside the room for a patient during an individualised session with allied health staff.
- Patients deemed at risk of sexual vulnerability are to be allocated to nursing staff of the preferred gender each shift where appropriate/practicable.
- Maximum visibility of patients on increased observation should be considered and room allocation prioritised where applicable.
- The special or companion is to adhere to standard and transmission-based precautions.

2. Request and Initiation of a Clinical Patient Special (including and AIN)

Request:

- A request for a special will be made by a member of the primary clinical treating team
- The Shift Coordinator is to then liaise with the ward or after hours SRN who will perform an assessment of the patient and recommend the category of special required and complete appropriate documentation.
- Consideration should be given to strategies such as patient allocation, ward acuity, skill mix and admission criteria to nursing special areas and ICU, dedicated HDU's.
- Review and Initiation:
- Be documented on AIN / PCA Observation & Assessment MR 864.1 or Request for Nurse Special MR 463 and in the patient's integrated notes
- Include indication(s) for a special i.e., clinical deterioration
- Include level of observation required i.e., at patients bedside, within room etc
- Identify designation of staff required to carry out the special observations i.e., mental health RN, RN general, AIN or security.
- Include any modifications to the initial request for a special
- Be reviewed at least once per shift (at least 2 hours prior to shift change).
- CNS, CNM,) To advise the Nurse Manager (NM) of the requirement. Out of office hours, the After Hours CNS/ CNM and the After Hours NM are to be notified.

- When approval for the Nurse Special or AIN / PCA or security surveillance has been obtained, an entry in the patient's integrated notes to be completed by the CNS/CNM identifying the outcome of the assessment and explanation of reason and category of special required.
- The NM is to refer to the Staffing and Agency Approval Process for Nursing if additional nursing staff is required for the patient special.

3. Request and Initiation of a Security special

A request for an adhoc security special may be made by the ward or after hours SRN, when a patient exhibits ongoing ASBD that places the patient, staff and others at extreme risk to their physical and mental wellbeing. The request will be discussed and approved by the appropriate Coordinator of Nursing (CoN) or Nurse Executive on call.

- Document in the integrated progress notes the rationale for the security special
- Complete [NMHS ADHOC Security Officer Request Form](#) (*add link to form here)
- Contact Security Control Centre on 6457 2501
- Forward completed form to
 - [NMHS Security Requests](#) (*add link to email address here)
 - Ward and or AH SRN, divisional Coordinator of Nursing and Co-Director
 - Commence Security Handover Form MR 689 (*new form will need to be linked when available)

4. Staff Re-allocation/ Cohorting

- Unless clinically indicated, or ward specific, staffing resources for the first special on the ward will be from existing staffing numbers.
- Where this is not practicable, consideration must be given to staff re-allocation and cohorting and is to be discussed with the ward or after hours CNS/CNM.
- Any further specialising requirements for the same patient or ward will require provision of additional staff.

5. Categories of Clinical Patient Special

1:1 Nurse Special

RN or EN required for the care of a physiologically, cognitively impaired, or at-risk patient with high acuity/complex care requirements which include:

- Continuous visual observation
- Frequent recording and interpretation of core physiological/cognitive observations (hourly or more frequent)
- Implementation of very demanding or frequent variations in treatment/care
- The need for frequent de-escalation of behaviours
- Assistance with activities of daily living (ADLs) and general/essential nursing care
- Regular re-orientation and diversion communication assisting with patient management and safety
- Implementing strategies agreed at an MDT/senior nurse level to address identified risks.
- Non-invasive ventilation for acute respiratory failure or acute pulmonary oedema
- Airway Management
- Intravenous medications requiring frequent observations
- Patient requires acute monitoring and is not suitable for admission to Intensive Care/ High Dependency Unit.

Mental Health Nurse special

Registered mental health nurse or comprehensive nurse required for all aspects of care including:

- specific assessment and management of acute/complex mental illness for mental health/behavioural issues requiring continuous evaluation, de-escalation and/or significantly challenging behaviours
- attending to patients' physical needs while working within scope of practice.

Patients requiring a mental health special in ED are to be reviewed periodically (each shift as minimum) by the psychiatric registrar and/or Consultant and ED CNS.

Patients requiring a mental health special in a ward at SCGH are to be reviewed by the Psychiatric Consultation Liaison Service [CL] at least daily.

Patients requiring a mental health special in a ward at OPH are to be reviewed by a Psychiatric Consultant from Osborne Lodge as required

- Following review, management plan to be amended where necessary.

Companion special

AIN / PCA is utilised for a physiologically **stable** patient i.e. patients with cognitive impairment requiring close observation and provision of essential/general nursing care under the direction and supervision of the Registered Nurse and is intended to support clinical staff in the care of patients

- For the purpose of this guideline AIN, PCA and agency Carers are referred to as Companions.
- The SRN is responsible for authorising and documenting rationale on the AIN / PCA Observation & Assessment Record MR 864 and defining if the AIN is for caring/safety requirements or behaviour concerns.
- The Behavioural Observation Form MR ### is commenced and will provide rationale for continuation of a companion and guides when escalation is required in relation to delirium / mental state and behavioural disturbance.
- When a patient has required a companion for a period exceeding 48 hours the relevant Coordinator of Nursing (CoN)Co-Director must be informed by the SRN.
- The SRN in consultation with the nurse will determine continuation of the need for observation by a companion. The SRN will contact the relevant Nurse Manager by 10:00am hours each day to confirm the planned reduction of companion observation over the next 24 hours (if reduction not anticipated, alternative plan).
- When the need for companion observation ceases, the Nurse Manager Patient Flow must be informed 3 hours prior to the commencement of next shift
- The companion will receive a handover each shift from the accountable nurse and both will sign the AIN / PCA Observation and Assessment Record at this time. The Companion will continue to fill out the observation form hourly.

6. Responsibilities

Staff undertaking specials will be:

- Directed to work within their scope of practice (and under the supervision of the RN where appropriate)
- Orientated to the ward including ward exits, alarms, processes, emergency procedures
- Orientated to use of relevant documentation, e.g., AIN / PCA Observation and Assessment Record MR, Observation and Response Chart.
- Orientated to patient specific management, e.g., diversional strategies

- Informed how to escalate identified needs and/or risks and who to inform – e.g., supervising RN / Shift Coordinator / CNS / CNM

Nurse is responsible for:

- Handing over relevant information to the team and specialising staff regarding the patient being looked after by the special
- Receiving updates on the patient's physiological/cognitive/mental state every 1 hour for patients receiving 1:1 observation and providing re-assessment as indicated
- Supporting the special in performing clinical duties as requested or where core requirements are out of scope of practice, modifying care interventions and changes to patient care plan as required
- Reporting any changes in condition to the shift coordinator and documenting any changes and/or decisions relating to patient care
- Completing the BOF as required or guided.
- Engaging with the patient to support patient centred management plans
- Overseeing clinical handover to the next shift and from outgoing special staff member to incoming special staff member including:
 - level of observation
 - reason for increased observations
 - levels of risk to self/others
 - risk reduction strategies being used to keep patient/self/team safe and reduce the possibility of adverse outcomes
 - what to do in the event of escalating behaviour/emergency situation
- Where appropriate, a handover from one nurse or staff member to another will involve the patient so that they are aware of what is being said about them. However, staff may need to exchange additional information separately.
- The nurse caring for the patient who has a companion or security officer must identify themselves to the companion at the commencement of the companion's shift, handover relevant care information and risk factors and sign and date that this has occurred on the AIN / PCA Observation and assessment Record or the Security Handover Form

AIN / PCA is responsible for:

- Assistance with ADLs and general/essential nursing care
- Continuous visual observation
- Regular re-orientation and diversion communication and activities assisting with patient management and safety
- Implementing strategies agreed at an MDT/Senior Nurse level to address identified risks
- Escalating concerns to overseeing nurse/shift coordinator.

Security Officer is responsible for:

- Receiving handover from primary nurse and complete Security Officer Form MR 689.
- Assist care under the guidance of the accountable nurse to ensure safe delivery of interventions being performed
- Assist with the delivery of diversional and engagement activities to reduce the risk of escalating behaviours
- Continuous visual observation and escalation of concerns to the accountable nurse or shift coordinator.
- Reports to the Security Team Leader on site

- Shift coordinator is responsible for: Introducing self to each companion or special and to ensure staff are orientated to ward exits, alarms, processes and emergency procedures
- Updating the ward or afterhours SRN each shift on the ongoing requirement or cessation of special
- Informing the staff member allocated to perform the special observations of their roles and responsibilities, lines of communication and procedures such as breaks/leaving the ward or unit
- Ensuring security/external security are employed to prevent imminent and serious injury to the patient and/or others, where required and completing handover between shifts and/or ongoing relevant handover during shifts as required.
- Ensuring adequate staff rotations for complex care patients for staff well being and delegation of appropriate meal break cover.
- Ensuring there is adequate supervision of the patient when special shift change occurs.

7. Special Review

The requirement for special or intermittent observation orders will be reviewed by the

- Patient's treating medical officer and the SRN as soon as practicable.
- Patients who are being specialised will be reviewed by the ward or afterhours SRN every shift and by medical staff at least once every 24 hours.
- Patients who are specialised for longer than 24 hours must be reviewed daily by the treating medical team and discussed with the Consultant. The review and outcome of all reviews must be documented in the patient's medical record.
- With regard to the special observation being reviewed the situation concerning constant observation can be reviewed and changed at any time providing that the medical officer and ward or afterhours SRN agree.
- The review and outcome of all reviews must be documented in the patient's integrated notes and Mechanical Restraint Authorisation and Observation ChartBOF/AIN record completed.
- If the special observation is only intended for specific times of day this must be documented clearly in the patient's integrated notes by ward or afterhours SRN
- As the patient's condition improves, staff are required to ensure there has been no undue delay in revising the patient's observation to the lowest level appropriate.
- Where there has been a marked deterioration in the patient's status, the next of kin/ carer will be notified.
- The decision to continue or cease the order for a Special must be documented in the patient's integrated notes and paper work accurately recording cessation date and time

8. Special Unavailable

Where the Special cannot be provided:

- The Shift Coordinator will escalate to the ward or afterhours SRN or delegate and inform the person who had made the initial request.
- The allocation of patients is to be reviewed to accommodate for the needs of the patient to be specialised and the patient acuity/case-mix of the ward and document in the patient's integrated notes.
- Where additional staff member(s) for a special either cannot be provided or be provided at the requested grade/skill level (i.e., AIN instead of RN):
 - alternative strategies are to be discussed

- the staff allocation of patients is to be reviewed and re-allocated to accommodate the needs of the patient to be specialised and the patient acuity/case-mix of the ward and document in the patient's integrated notes.

The following must be clearly documented within the inpatient progress notes:

- Name/role of person informing of inability to provide Special
- All escalations and outcomes to ward or afterhours SRN
- Escalation to treating medical team/mental health team
- Any mitigation strategies employed to reduce risk to patient concerned.

Special Request Declined

If the request for a Special is not approved, staff are to continue to monitor the patient's condition closely and regularly review risk assessment and effectiveness of risk minimisation strategies.

- The decision must be clearly documented in the patient's integrated notes including the reasons why the request was declined and by whom.
- This outcome is to be escalated to the person who had made the initial request or delegate.

9. Discontinuation of a Special

- The ward or afterhours SRN / treating MO have the delegated authority to discontinue a non-mental health special following patient reassessment which is to be documented in the patient's integrated notes and communicated to the shift coordinator.
- Mental health specials in a non-authorized hospital require consultation with mental health staff and specialty treating teams before the removal of the special.
- If a special is no longer required, the shift coordinator / ward or afterhours SRN to inform Nurse Manager Patient Flow to ensure that any staff booked are cancelled in a timely manner, and the rationale should be clearly documented in the patient record.

10. Incident Reporting

- Any identified risks of aggression or self-injury are to be documented in the patient integrated notes and reflected in the relevant care plan
- Any incidents are to be documented in the Datix CIMS by relevant clinician/ supervising RN where appropriate

Compliance, monitoring and evaluation

- All additional staff for patient specials are recorded within the NM process for nursing hours per patient day (NHPPD).
- Every fortnight the NM will reconcile their NHPPD used with the recommended allocation for that ward / unit.
- Nursing Executive roles will check and discuss any issues relating to the NM reporting of NHPPD for the areas of responsibility.
- Nursing Executive Committee monitors the NHPPD reports issued each month.

Related Documents

[Assistant in Nursing Policy \(health.wa.gov.au\)](http://health.wa.gov.au)

[Delirium Clinical Care Standard](#)

[SCGOPHCG Clinical Handover – Communicating for Safety](#)

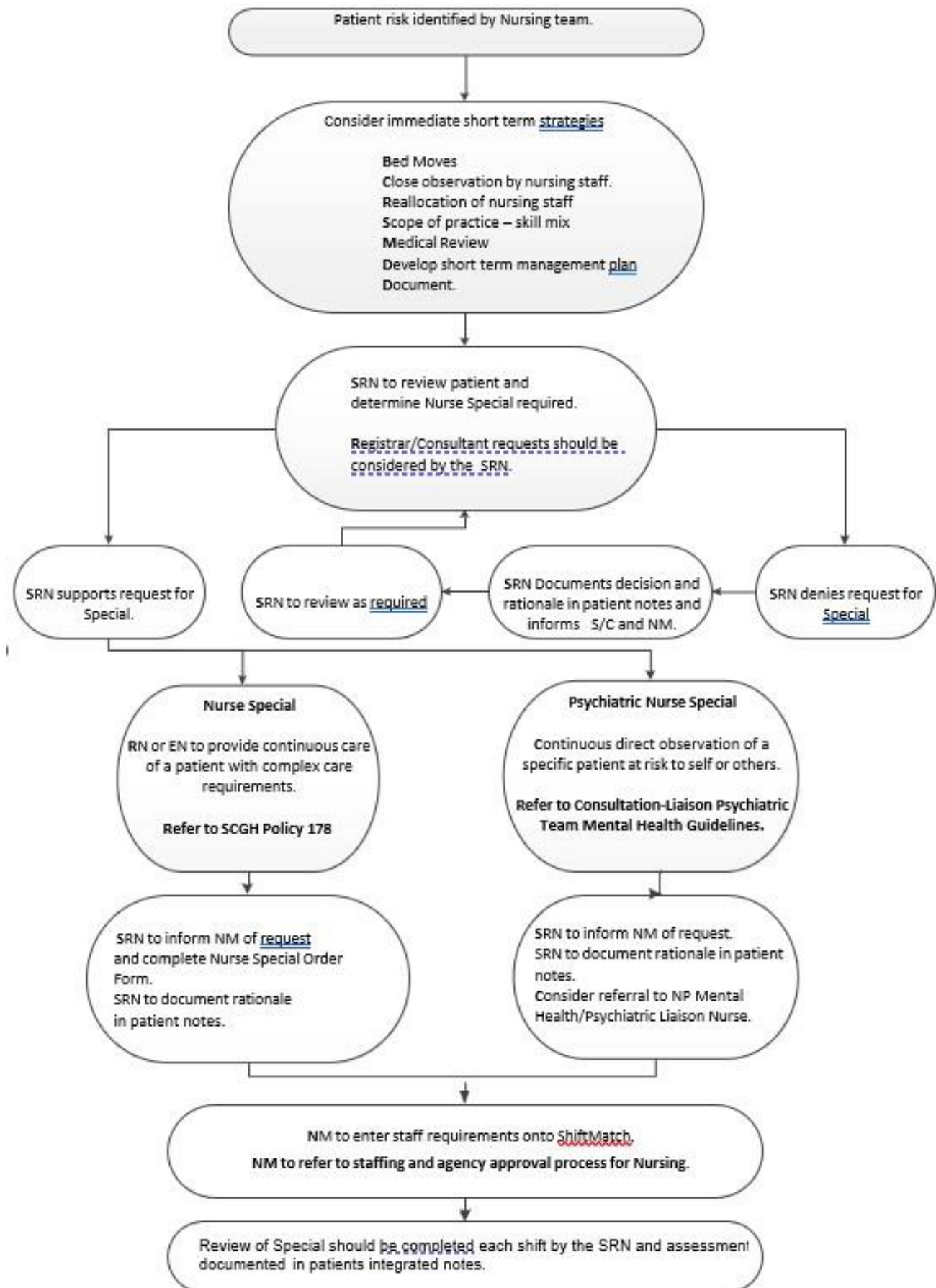
SCGOPCHG Use of Restrictive Practices in a Non-Authorised Setting

Authorisation & Version Control

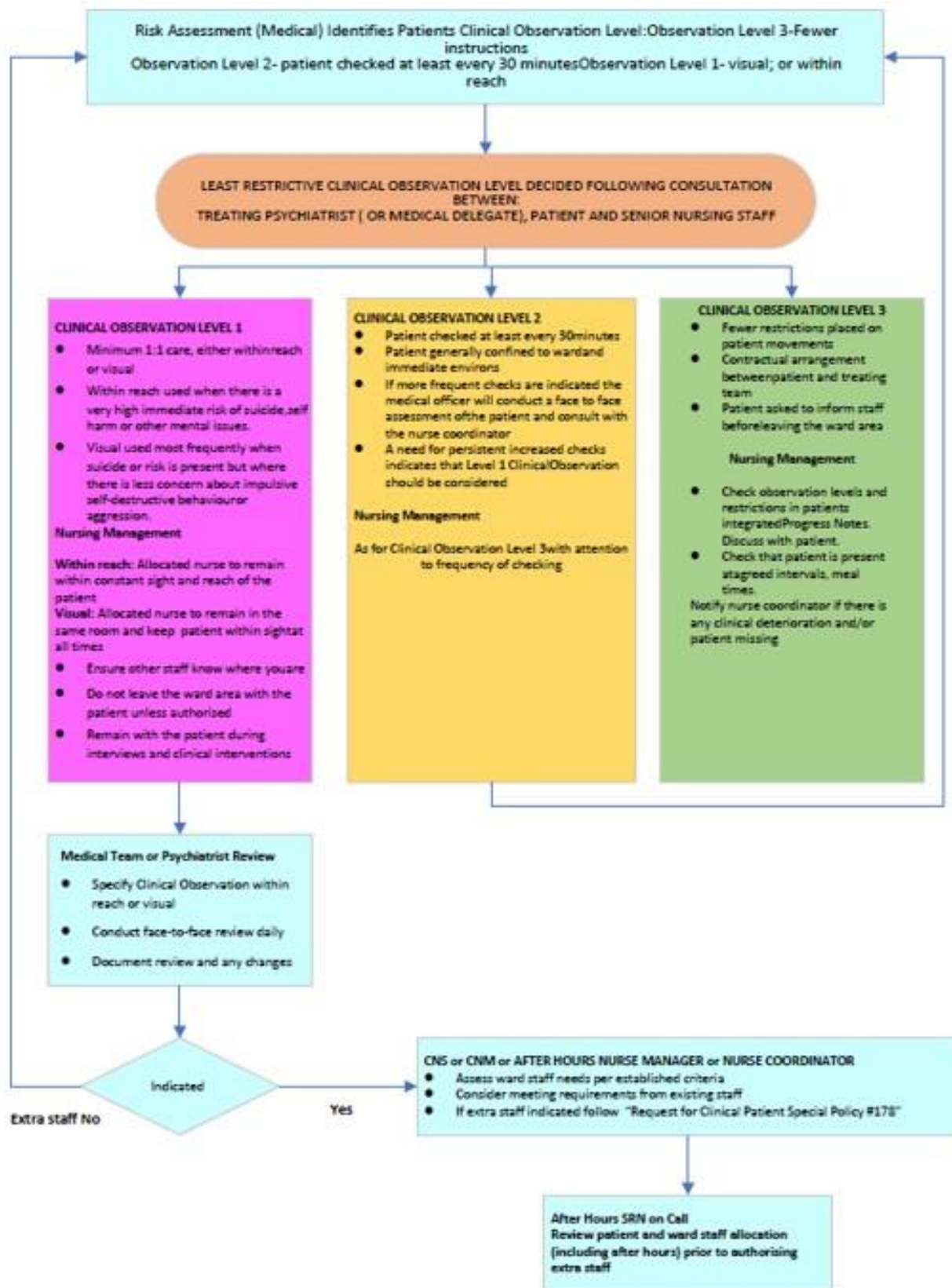
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For full details regarding, development, review, consultation, approval, endorsement - contact the Policy Officer for the submission form

Appendix 1: Assessment and Management of Specials Flowchart



Appendix 2: Mental Health Observation Guidelines



Appendix 3: Nursing Relief / Additional Staff Approval Process

Note: Ward/Department profiles are set from the allocated Nursing Hours per Patient Day (NHpPD) and should be managed accordingly.

NURSING		
Monday – Sunday (AM & PM)	APPROVAL	PRIORITISATION
Up to NHpPD / profile per Ward / Department	Nurse Manager / Clinical Nurse Manager	1. Resource balance staff from areas with available staff
1 above NHpPD / profile per Ward / Department (including Special requests)	Nurse Manager / Clinical Nurse Manager with notification to /Nurse Co-Director for the Division	2. RNS nurses 3. Part time nurses / Casual 4. Agency nurses
2 or more above NHpPD / profile per Ward / Department (including Special requests)	<u>In Hours:</u> NM/CNM to seek approval from /Nurse Co- Director for the Division <u>After Hours:</u> NM/CNM with approval from the Nurse Executive on call for Beds & Staffing	5. Full-time nursing staff doing overtime 6. Source other available RN's <i>**RN & EN shortages should be backfilled by RN/EN's</i>
Monday – Sunday (NIGHT DUTY)	APPROVAL	PRIORITISATION
1. above NHpPD / profile per Ward / Department (including Special requests)	A/H Nurse Manager	• Resource balance staff from areas with available staff
2. or more above NHpPD / profile per Ward / Department (including Special requests)	After Hours NM/CNM with approval from the Nurse Executive on call for Beds & Staffing	• RNS nurses • Part time nurses / Casual • Agency nurses • Full-time nursing staff doing overtime

NURSE PSYCHIATRIC SPECIALS		
Monday – Sunday (all shifts)	APPROVAL	PRIORITISATION
Psychiatric Specials are NOT contained within numbers The patient must meet the following criteria to be considered a “Psychiatric Special”; • Be under the care/review of a psychiatric team • The requirement for a “Special” is documented in the patient’s medical record by either a Psychiatrist, CNC, or Mental Health Nurse Practitioner	NM/CNM / After Hours NM/CNM	1. Utilise part time staff if available, otherwise: 2. Use existing extra ward staff that are comprehensive, or psych. trained. 3. Use RNS or casual staff who are comprehensive or psych trained 4. Use agency nursing staff that are comprehensive, or psych trained 5. Use overtime staff that are comprehensive, or psych. trained 6. Use agency AIN's to backfill while awaiting comprehensive or psych. trained

ADDITIONAL AIN (Companion)		
Monday – Sunday (all shifts)	APPROVAL	PRIORITISATION
- The patient is generally physiologically stable requiring close observation and provision of essential/general nursing care under the direction and supervision of the Registered Nurse - For wards/Departments where AINs form part of the establishment: first special/companion to be contained by that AIN - AIN's should not be used for 'High Acuity' - Type of AIN distinguished for Carer or behavioural reasons	- Consider if the patient requires nursing intervention. - The SRN will complete a comprehensive nursing assessment on the AIN/PCA Observation & Assessment Record MR 864.1 - First companion contained by utilising the ward AIN - Meal relief for the companion is allocated within the ward staffing - When a patient has required a companion for a period exceeding 48 hours the relevant Coordinator of Nursing/Co-Director must be informed by the SRN	- All companion specials to be reviewed by the SRN each shift to assess the requirement for continuation on the next shift - The SRN will contact the relevant Nurse Manager by 10:00am each day to confirm companion observation requirements over the next 24 hours - When the need for companion observation ceases, the Nurse Manager, Patient Flow must be informed as soon as possible - Ward AIN's are to be backfilled if deemed appropriate for clinical safety
SCGH up to 20/ OPH up to 5	NM/CNM	For wards/department where AIN's form part of the establishment and are undertaking a companion special for the shift will not be counted in the 20 or 5
SCGH above 20/OPH above 5	In hours: NM/CNM seek approval from the CDirector /Nurse Co-Director for the Division After Hours: NM/CNM seeks approval from the Nurse Executive on call	

Appendix 4: RoStar Job Designation Codes

Job Designation Description	Designation code	Definition
AIN Carer	AINC	AIN is suitable to assist with activities of daily living to maintain safety within the hospital environment, i.e. wandering and needing redirection including falls risk. This can also include patient escorts and/or transfers.
AIN Behaviour	AINB	AIN required to engage in positive engagement strategies due to behavioural disturbances outlined on the AIN observation form.
AIN High Acuity	AINH	AIN required to assist the ward with general nursing due to the higher-than-normal acuity on the ward.
AIN Mental Health Nurse Backfill	AMBF	Unable to provide a Mental Health Nurse Special and is suitable to be backfilled by an AIN, or patient is held under doctrine of necessity, not on MHA forms and is not medically cleared.
Clinical Nurse Special	SPCL	Patient requires intensive nursing care due to deterioration or clinical care requirements, including nutritional restoration program.
Mental Health Nurse Special	SPMH	A patient is placed under the Mental Health Act 2014 and requires a mental health trained special.
High Acuity Registered Nurse	HARN	The increased clinical requirements of the ward required to mitigate clinical risk.